

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 2 Randomized Clinical Study of MK-4280A (Coformulated Favezelimab [MK-4280] Plus Pembrolizumab [MK-3475]) Versus Physician's Choice Chemotherapy in PD-(L)1-refractory, Relapsed or Refractory Classical Hodgkin Lymphoma (KEYFORM-008)
Short Title:	A Study of Coformulated Favezelimab/Pembrolizumab (MK-4280A) Versus Physician's Choice Chemotherapy in PD-(L)1-refractory, Relapsed or Refractory Classical Hodgkin Lymphoma (MK-4280A-008)
Protocol Number:	273647628979273717
NCT Number:	NCT05508867
Sponsor Name:	Merck
Investigational Product:	gemcitabine
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression Free Survival (PFS) per Lugano Response Criteria as Assessed by investigator: PFS is defined as the time from randomization to the first documented disease progression per Lugano criteria 2014 as assessed by investigator or death due to any cause, whichever occurs first.
Secondary Obj.:	1. Overall Survival (OS) 2. Objective Response Rate (ORR) per Lugano Response Criteria as Assessed by investigator 3. Duration of Response (DOR) per Lugano Response Criteria as Assessed by investigator

2.2 Background & Rationale

Researchers are looking for a way to treat classical Hodgkin lymphoma (cHL) that is relapsed (the cancer has come back after treatment) or refractory (current treatment has stopped working to slow or stop cancer growth). Researchers want to learn if people who receive coformulated favezelimab/pembrolizumab (MK-4280A) live longer without the cancer getting worse compared to those who receive chemotherapy.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: ACTIVE_NOT_RECRUITING
Target N:	203

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

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- * Has histologically confirmed diagnosis of classical Hodgkin lymphoma (cHL) that is 2-fluorodeoxyglucose-avid (FDG-avid).
- * Has relapsed (defined as disease progression after most recent therapy) or refractory (defined as failed to achieve CR or PR to most recent therapy) cHL and exhausted all available treatment options with known clinical benefit.
- * Has progressed on treatment with an anti-PD-(L)1 monoclonal antibody (mAb) administered either as monotherapy or in combination with other checkpoint inhibitors or other therapies.
- * Submits an archival (?5 years) or newly obtained tumor tissue sample which has not been previously irradiated.

4.2 Exclusion Criteria

- * Diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy or any other form of immunosuppressive therapy.
- * History of central nervous system (CNS) metastases or active CNS involvement.
- * Has an active autoimmune disease that has required systemic treatment in past 2 years except replacement therapy.
- * History of (noninfectious) pneumonitis/interstitial lung disease that required steroids or has current pneumonitis/interstitial lung disease.
- * Has an active infection requiring systemic treatment.
- * History of hemophagocytic lymphohistiocytosis.
- * Has an active seizure disorder that is not well controlled.
- * Has clinically significant (ie, active) cardiovascular disease.
- * Received prior systemic anticancer therapy including investigational agents within 4 weeks before randomization.
- * Received prior radiotherapy within 2 weeks of start of study intervention or radiation related toxicities requiring corticosteroids.
- * Has not adequately recovered from major surgical procedure.
- * Known additional malignancy that is progressing or has required active treatment within the past 3 years.
- * History of human immunodeficiency virus (HIV).
- * Has had an allogeneic hematopoietic stem cell or solid organ transplantation within the last 5 years.

11. Study Identifiers & Governance

Primary ID: 273647628979273717

Registry Number: NCT05508867

Sponsor: Merck

Study Title: A Study of Coformulated Favezelimab/Pembrolizumab (MK-4280A) Versus Physician's Choice Chemotherapy in PD-(L)1-refractory, Relapsed or Refractory Classical Hodgkin Lymphoma (MK-4280A-008)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____